



OFFICIAL

NMHS MHS POLICY

Medication: Off-Label Prescribing Including Prescribing Greater than Maximum Dose

Scope (Audience)	All NMHS Mental Health Services (MHS) Clinical Staff
Scope (Area)	All NMHS MHS - Adult
Summary	To ensure consistent and evidence-based practice for prescribing and administering medication outside of the registered product information. For the purposes of this policy, " <i>medication</i> " refers to both psychotropic and non-psychotropic medication.
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Medication Safety (Std 4) Comprehensive Care (Std 5) Communicating for Safety (Std 6)
Policy Sponsor	Director of Medical Services & Chief Pharmacist MHSDHS
Policy Contact	Chief Pharmacist MHSDHS
Version	V5.0
Review Date	23/03/2028

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

To ensure consistent and evidence-based practice for prescribing and administering medication outside of the registered product information. For the purposes of this policy, “medication” refers to both psychotropic and non-psychotropic medication.

2. Background

Off-label prescribing refers to the prescription of a medicine that is used in a manner other than that specified in the Therapeutic Goods Administration (TGA) approved product information. This includes when the medicine is prescribed:

- for another indication
- at a different dose
- via an alternate route of administration
- for a patient of a demographic outside the registered use.

Off-label prescribing occurs on a spectrum. When there is high quality evidence to support use or prescribing is very slightly out of the realm of license and has a favourable risk/benefit ratio, off-label prescribing is generally considered routine or low risk. Where there is low quality evidence to support the use and/or prescribing is very far out of the realm of license and/or the risk/benefit ratio is less favourable, prescribing is generally considered as exceptional use.

In all situations, prescribers must demonstrate that other appropriate treatment options have been explored and deemed unsuitable, that the potential benefits outweigh the potential harm, that the risks have been evaluated and methods for assessing efficacy of the off-label prescribing are planned.

For patients under the age of 18 please refer to the [WA Child and Adolescent Health Services Use of Medications for other than Manufacturer’s Approved Indications Policy](#).

3. Risk

Failure to comply with the principles outlined in this policy may expose the patient to risk of unnecessary harm or lack of benefit, the clinician to professional liability and the health service to financial and legal ramifications.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Policy Principles – Prescribing Off-Label Medications

The TGA approved indication, dose, route, and patient factors for a medication can be found on the TGA website ([Product Information | Therapeutic Goods Administration \(TGA\)](#)) and in the

product information accessible from AusDI. Prescribing must comply with the product information unless prescribing takes place in accordance with this policy.

- Prescribers must also adhere to the requirements of the [NMHS MH Formulary and Non-Formulary Prescribing Policy](#). Medicines that are listed on the [WA Statewide Medicines Formulary](#) (SMF) may in fact be 'off-label' with reference to the manufacturer's instruction.
 - An Individual Patient Approval (IPA) application form must also be completed and approved for medications not on the SMF as per the [NMHS MH Formulary and Non-Formulary Prescribing Policy](#).
- Off-label prescribing may only occur on the authority of a consultant psychiatrist or an appropriate specialist.
- Informed consent from the patient and/or guardian must be obtained and documented as outlined in the [WA Health Consent to Treatment Procedure MP0175/22](#).
 - If informed consent cannot be obtained from an involuntary patient, a second opinion from another consultant psychiatrist must be obtained.

For complex situations or for medication doses or indications with poor evidence for use the Head of Clinical Service and/or the Chief Pharmacist should be involved in the decision-making process.

The rationale and plan for treatment must be documented in the patient's integrated progress notes and must include:

- the reason for the off-label medication, and evidence supporting its use
- other strategies that have been tried and failed
- a summary of risks that have been evaluated
- discussion with the patient and/or guardian, including patient and/or guardian consent or a second opinion for involuntary patients unable to consent
- physical examination and investigations
- how treatment will be evaluated and monitored
- review time frames and the duration of treatment

4.1 Prescribing Greater Than Maximum Dose

- Prescribing greater than maximum dose is a type of off-label prescribing that refers to the prescription of a single medication that exceeds the licensed maximum dose in Australia.
- It may also refer to the prescription of two (or more) medications from the same class (e.g. antipsychotics) such that when the individual doses are expressed as a percentage of their maximum licensed dose, the sum of the percentages is greater than 100%.
 - This is considered 'off-label' prescribing for the purposes of this policy even if one or all medications assessed individually do not exceed the licensed dose.
- Of particular relevance to mental health are antidepressant and antipsychotic classes of medication.
- A documented plan for closer monitoring and identification of adverse effects is essential as many adverse effects are dose related, and therefore prescription of high or combined doses are likely to result in an increased adverse effect burden.

4.2 Consumer Information

Patients and guardians must be provided with sufficient verbal and written information regarding the treatment to enable informed decision making. Carers must be provided with information to facilitate monitoring and identification of side effects.

It is important to counsel patients, guardians and carers on any inconsistencies in written information provided to prevent unnecessary concern or confusion.

The Choice and Medication website may be used as a resource to provide patients with information on psychotropic medications. Many of the medication leaflets refer to common off-label indications.

Formulary One has simplified Medication Information Leaflets (MILs) for a range of non-psychotropics medications.

4.3 Transfer of Information to Healthcare Providers

At transfers of care, receiving healthcare providers must be fully informed and appropriate documentation received that an off-label medication has been prescribed, the duration of treatment and any ongoing monitoring requirements.

5. Roles and Responsibilities

All NMHS staff	Are responsible for: • Having knowledge of the policy documents relevant to their roles and how to access these documents. • Complying with mandatory requirements outlined in these documents. • Assisting with policy development/reviews wherever required. • Reporting to their immediate line manager any potential non-compliance or implementation issues.
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6. Monitoring and Evaluation

Compliance with this policy is mandatory. Compliance with this policy will be assessed via the Medication Safety audit cycle by:

- Drug Utilisation Audit of Higher than Maximum Licensed Dosing (NMHS MH pharmacy)

This policy will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed 3 years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

7. Related Documents

WA Health mandatory policies	• WA Statewide Medicines Formulary • MP 0175/22 Consent to Treatment Policy • MP 0175/22 Consent to Treatment Procedure
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NMHS MHS Policy documents	• NMHS MH Formulary and Non-Formulary Prescribing Policy
NSQHS Standards	• National Safety & Quality Health Standards Ed 2 Nov 2017
Other	• CAHS Use of Medications for other Manufacturer's Approved Indications Policy • CATAG: Guiding principles for the quality use of off-label medicines • AusDI • RANZCP Professional Practice Guideline 4: Off-label prescribing in psychiatry

8. Document Control

Version	Policy author	Summary of changes	Approval date
V1.0	MHEG	Meetings Minutes 134/15	17/06/2015
V2.0	Executive Director	Patrick Marwick	17/08/2015
V3.0	NMHS MH Chief Pharmacist	Required review; New template; Adding information on Off-label prescribing; hyperlinks updated.	11/08/2022
V4.0	NMHS MH A/Medicines Information Pharmacist	Updated to reflect current resources, available medication, systems and technology	18/07/2025
V5.0	NMHS MH DTC	Required review, Updated maximum doses	23/03//2026

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community. ISD: 5303.

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